Clinical Dashboard

Real-time neonatal health monitoring and AI diagnostics.

Behavioral State (NBAS)

RESPONSIVE

CURRENT STATE
QUIET ALERT
NBAS CODE: 4

EYES OPEN — YES
SPONTANEOUS MOVEMENT — YES
STABILITY — STABLE

BRAZELTON NEONATAL SCALE

Neonatal Skin & Jaundice Assessment

CRITICAL ALERT

MAPPING:

1. BILIRUBIN
17.5 mg/dL
SEVERITY 70%

2. KRAMER ZONE
ZONE 4

3. DENSITY
High Intensity

SCLERAL ICTERUS
Visible

CONFIDENCE
HIGH

CRITICAL STATUS
High Kernicterus Risk Detected

DIAGNOSTIC REGION
Palms and Soles

Skin Lesion Analytics

URGENT REVIEW

SURFACE MAPPING
TRUNK FACE



HISTOGRAPHIC PROJECTION

CLINICAL ASSESSMENT
LESIONS DETECTED
2 ANATOMICAL SITES FLAGGED

DISTRIBUTION
SCATTERED

DOMINANT COLOR
RED/PURPLE

SUSPECTED ETIOLOGY
THROMBOCYTOPENIA

SIZE CATEGORY
PINPOINT

PETECHIAE	DETECTED	PURPURA	CLEAR
ECCHYMOSIS	CLEAR	BLUEBERRY MUFFIN	CLEAR

LIVE CLINICAL FEED
ACTIVE ANALYSIS

Respiratory Analytics

BRADYPNEA ALERT

RESP RATE
28 BPM
SLOW_REGULAR PATTERN
WINDOW: 15S

SILVERMAN-ANDERSEN SCORE

TOTAL SCORE
15

SEVERITY
MODERATE RESPIRATORY DISTRESS

UPPER CHEST	5
LOWER RETRACTIONS	10
XIPHOID RETRACTION	10
NASAL FLARING	10
EXPIRATORY GRUNT	No Grunt

AI RESPIRATORY MONITORING ACTIVE
TACHYPNEA BRADYPNEA APNEA

NIPS Pain Assessment

PAIN INDICATED

TOTAL SCORE
3
MILD PAIN

NIPS PARAMETERS

FACIAL EXPRESSION	1
CRY	1
BREATHING PATTERN	0
ARMS	0
LEGS	0
STATE OF AROUSAL	1

NEONATAL INFANT PAIN SCALE
CLINICAL ACTION INDICATED

Gastrointestinal Assessment

GASTRO ALERT

ABDOMINAL CONTOUR
DISTENDED
AI SHAPE ANALYSIS

ABDOMINAL DISTENSION	DETECTED
VISIBLE VENOUS ENGORGEMENT	DETECTED
SHINY ABDOMINAL WALL	NEGATIVE

NEC SCREENING PROTOCOL ACTIVE

Neurological Assessment Suite

FULL KINEMATIC & NEURO-DIAGNOSTIC STREAM

NEURO-CRITICAL

MUSCLE TONE & POSTURE
NORMAL
POSTURAL RESISTANCE LEVEL

FROG-LEG POSTURE
W-ARM POSTURE

MOVEMENT QUALITY (PRECHTL)
FIDGETY: ⊘ ABSENT

POOR REPERTOIRE
"Monotonous movements with reduced complexity and variation"

DETAILED KINEMATIC BREAKDOWN

SEIZURE/JITTER DIFFERENTIAL

RHYTHMIC TREMOR	●	OCULAR DEVIATION	●
LIP SMACKING	●	APNEA WITH TREMOR	●
SUPRESSIBLE	○	JITTERINESS SUSPECTED	○

RISK FACTOR: HIGH
FREQ PATTERN: HIGH

LIMB SYMMETRY CONTROL
ASYMMETRICAL — RIGHT_UPPER
ERB'S PALSY (WAITER'S TIP)
Significant reduction in spontaneous movement of the right arm

POSTURE ASSESSMENT (HEAD/NECK)
TORTICOLLIS ● OPISTHOTONUS ○
TILT DEGREES: 15°
DIRECTION: LEFT

ACTIVE NEUROLOGICAL INTERVENTION ADVISED
INITIATE NEONATAL NEURO-PROTOCOL

Craniofacial Morphology

MORPHOLOGY ALERT

FACIAL SYMMETRY
ASYMMETRICAL

DYSMORPHIC FEATURES
SUSPECTED

MORPHOLOGICAL DETAILS

EAR POSITION	EYE SPACING	FISSURE SLANT	NASAL BRIDGE
Low_set	Hypertelorism	Up-Slanting	Depressed

" Mild facial asymmetry with low-set ears and widened eye spacing detected. Genetic consultation recommended. "

AI DYSMORPHOLOGY SCREENING V1.2
CLINICAL REVIEW ADVISED

IMNCI Danger Signs

WHO INTEGRATED MANAGEMENT PROTOCOLS

IMMEDIATE DANGER

ROUTINE: LETHARGY / UNCONSCIOUS — NOT ACTIVE	ROUTINE: SEVERE CHEST INDRAWING — NOT ACTIVE	CRITICAL: SUSTAINED TACHYPNEA — DETECTED
ROUTINE: CENTRAL CYANOSIS — NOT ACTIVE	ROUTINE: CONVULSIONS / SEIZURE — NOT ACTIVE	

WHO NEONATAL CARE STANDARDS
IMMEDIATE ESCALATION REQUIRED

Clinical Decision Support

LAST ASSESSMENT: REAL-TIME

IMMEDIATE ESCALATION

DIAGNOSTIC REASONING

"Infant presents with significant respiratory distress (Silverman-Andersen Score: 15) and sustained tachypnea. Jaundice has progressed to Kramer Zone 4 with TSB estimates >20 mg/dL. These findings, combined with neurological irritability, necessitate immediate ICU escalation."

RECOMMENDED ACTIONS

- IMMEDIATE NICU TRANSFER
- START INTENSIVE PHOTOTHERAPY
- OXYGEN THERAPY & SEPSIS WORKUP
- SENIOR PEDIATRIC CONSULT

ACTIVE TRIGGERS
SEVERE RESPIRATORY DISTRESS | SUSPECTED SEPSIS | CRITICAL HYPERBILIRUBINEMIA

AI CLINICAL INSIGHT ENGINE V4.2.0
ACKNOWLEDGE EMERGENCY

PATIENT PROFILE

Ward 4B

Godwin Amadi
ID: b6cd34-2025

AGE: 4 Days
WEIGHT: 3.2 kg

SESSION CONTEXT

Analyzed At: 15:30
Lighting: CLINICAL OVERHEAD
Position: SUPINE

CLINICAL GUIDANCE

"Infant presents with significant respiratory distress (Silverman-Andersen Score: 15) and sustained tachypnea. Jaundice has progressed to Kramer Zone 4 with TSB estimates >20 mg/dL. These findings, combined with neurological irritability, necessitate immediate ICU escalation."

RECOMMENDED ACTIONS

- Immediate NICU Transfer
- Start Intensive Phototherapy
- Oxygen Therapy & Sepsis Workup
- Senior Pediatric Consult